Fax cover sheet

Willowbrook Bay Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Teresa Valdez, LCSW
Phone: 555-0141
Date: 09/25/2026 08:15
Pages (including cover): 14
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

48M admitted with acute alcoholic pancreatitis, now resolved and tolerating diet. Completed inpatient withdrawal monitoring. Requesting recuperative care start 09/25/2026.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Willowbrook Bay Medical Center (fictional)
Department/Floor: 6 West Medicine
Social Worker/Case Manager/Nurse: Teresa Valdez, LCSW
Phone: 555-0141
Email: tvaldez@willowbrookbay.example
Nursing Station: 555-0142
Authorized By: Dr. N. Okafor

Patient Information

Name: Raymond Doe
MRN: WB-6613027
Birth Date: 05/14/1978
Gender: M
Ethnicity: White
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Acute alcoholic pancreatitis, alcohol use disorder
Admit Date: 09/22/2026
Any Wounds: Yes [ ] No [X]
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No [X]
PPD/TB Test or Chest X-Ray: Date: 09/22/2026 Result: CXR neg

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [ ] No [X]
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Any limitations or challenges: No [ ] Yes [X]

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Raymond Doe
DOB: 05/14/1978
Member ID: OCH-40217736

Referring Provider

Facility: Willowbrook Bay Medical Center (fictional)

Clinical

Primary Diagnosis: Alcohol-induced acute pancreatitis without necrosis or infection
ICD-10: K85.20, F10.20
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 09/25/2026
Requested Days: 21
Admit Date: 09/22/2026
Clinical summary: pancreatitis resolved, tolerating diet, independent ADLs, oral meds only.

Plan Use Only

Status: Pending review

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Patient face sheet

Willowbrook Bay Medical Center (fictional) - PATIENT FACE SHEET

Patient: Raymond Doe
MRN: WB-6613027
Account #: 99613027
DOB: 05/14/1978
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 09/22/2026 04:30 (ED arrival 09/21/2026 23:40)
Admit Type: Inpatient
Unit/Bed: 6W-614
Attending: Nnamdi Okafor, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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Emergency department note

EMERGENCY DEPARTMENT PROVIDER NOTE

Arrival: 09/21/2026 23:40 via EMS (called by outreach worker)

HPI

48 y/o M, unhoused, with alcohol use disorder, presents with severe epigastric pain radiating to back x 1 day, nausea, vomiting. Pt intoxicated on arrival. Reports drinking about 12 beers daily; last drink 09/21/2026 around 22:00.

Exam

T 99.1F HR 112 BP 148/90 RR 20 SpO2 96% RA
Alert, smells of alcohol, slurred speech. Epigastric tenderness, no rebound. No jaundice.

Data

BAL 228 mg/dL (09/22 00:10)
Lipase 1,240 U/L H, AST 88 H, ALT 46 H, T bili 0.9, Glu 138, Ca 9.0, TG 210
CT abdomen with contrast: acute interstitial edematous pancreatitis, no necrosis, no fluid collection; no gallstones; liver without cirrhotic morphology

ED Course / MDM

Acute alcoholic pancreatitis and acute alcohol intoxication. IV fluids, IV analgesia, antiemetic. Admit to medicine with CIWA monitoring.
Signed: P. Lindqvist, MD 09/22/2026 03:55

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL

Admit order: 09/22/2026 04:30

CC

Epigastric pain, alcohol intoxication

HPI

48 y/o M with alcohol use disorder, presented 09/21 23:40 intoxicated (BAL 228) with acute pancreatitis (lipase 1,240). First episode of pancreatitis. No prior withdrawal seizures or DTs. Unhoused x 2 yrs, sleeps in a park in Example City.

PMH

- Alcohol use disorder, moderate
- No psychiatric history
- No diabetes

Meds PTA

None

Allergies

NKDA

Social

EtOH as above. Tobacco 1/2 ppd. Denies drug use.

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History and Physical - page 2

Exam

T 99.0F HR 108 BP 144/88 RR 18 SpO2 96% RA
Mild tremor, not diaphoretic. AOx4. Abdomen tender epigastrium.

Assessment/Plan

1. Acute alcoholic pancreatitis, mild (no necrosis) - NPO, IV LR, IV analgesia, advance diet as tolerated
2. Acute alcohol intoxication, at risk for withdrawal - CIWA-Ar q4h, lorazepam symptom-triggered (CIWA >= 8)
3. Thiamine, folic acid, multivitamin
4. Transaminitis, likely alcohol related - RUQ US
5. Addiction medicine and SW consult

Signed: Nnamdi Okafor, MD 09/22/2026 06:10

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CIWA-Ar flowsheet

CIWA-Ar FLOWSHEET

Date/Time | CIWA-Ar | Lorazepam given | RN
09/22 04:00 (ED) | 9 | 1 mg IV | PL
09/22 12:00 | 10 | 1 mg IV | MS
09/22 20:00 | 8 | 1 mg PO (LAST DOSE) | MS
09/23 04:00 | 6 | none | RB
09/23 12:00 | 5 | none | MS
09/23 20:00 | 4 | none | MS
09/24 04:00 | 3 | none | RB
09/24 12:00 | 2 | none | MS
09/24 20:00 | 2 | none | MS
09/25 04:00 | 1 | none | RB

Protocol: lorazepam for CIWA >= 8. CIWA-Ar below 8 on every check since 09/23 04:00. No seizure activity. No hallucinations.

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#4

Date: 09/25/2026 07:30
S: No abdominal pain. Ate full low-fat breakfast and dinner yesterday, no nausea. Sleeping well.
O: T 98.2F HR 78 BP 128/80 RR 16 SpO2 98% RA. No tremor, no diaphoresis. AOx4. Abdomen soft, non-tender.
Lipase 09/24: 96. CIWA-Ar 1 at 04:00. No benzodiazepines since 09/22 20:00.
A/P:
1. Acute alcoholic pancreatitis - resolved; tolerating low-fat diet; no IV fluids since 09/23
2. Alcohol withdrawal - none since 09/23; more than 72 hours since last drink (09/21 22:00)
3. AUD - acamprosate started 09/24; pt accepted outpatient treatment
4. All meds now oral. No IV access needed; PIV removed 09/24.
Medically stable for discharge to recuperative care today.
Signed: Nnamdi Okafor, MD

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Physical and occupational therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 09/24/2026 10:30
Bed mobility: independent
Transfers: independent, no device
Gait: 400 ft independent, no assistive device, steady, no loss of balance
Stairs: 12 steps independent with rail
Recommendation: no PT needs, no DME. Safe for discharge.
Signed: J. Whitaker, PT, DPT

OCCUPATIONAL THERAPY EVALUATION

Date: 09/24/2026 13:00
Grooming, UB/LB dressing: independent
Toileting and toilet transfer: independent
Shower: independent, gathered own supplies, stood for shower
Cognition: AOx4, follows multi-step commands, good safety awareness
Recommendation: independent with all ADLs. No OT needs.
Signed: R. Banerjee, OTR/L

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Social work note

SOCIAL WORK / ADDICTION MEDICINE NOTE

Date: 09/24/2026 15:20
Pt calm and cooperative, AOx4. Denies SI/HI. No psychosis. No psychiatric history.
No conservatorship. Not enrolled in FSP.
Pt states he wants to stop drinking after this hospitalization. Residential treatment offered; pt prefers outpatient.
Pt ACCEPTED outpatient program: Example Bay Outpatient Recovery (fictional) IOP, intake appointment 09/29/2026 10:00, phone 555-0147.
Pt agrees to abstain from alcohol while in recuperative care and to attend outpatient follow-up.
Pt interested in recuperative care; understands sober-living expectations.
Signed: Teresa Valdez, LCSW

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Nursing shift note

NURSING SHIFT NOTE

Date: 09/25/2026 06:30
Night uneventful. Pt slept, ambulated in hallway independently x3.
Self-administered meds with RN observation (teach-back for acamprosate and thiamine done; pt verbalized dose and schedule).
Continent of bowel and bladder, toileting independently. Showered independently 09/24 evening.
Calm and cooperative with staff. No isolation precautions.
Signed: R. Bautista, RN

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Labs and vitals

LABS

09/22 00:10 | BAL 228 mg/dL | Lipase 1,240 H | AST 88 H | ALT 46 H | Glu 138 | Ca 9.0
09/22 06:00 | Beta-hydroxybutyrate 0.2 (neg) | Mg 1.8 | Phos 3.0 | INR 1.0 | Alb 3.9
09/24 05:00 | Lipase 96 | AST 42 | ALT 38 | Glu 94 | Cr 0.8 | WBC 7.6 | Hgb 13.8
09/24 HbA1c: 5.4%
09/22 MRSA nares PCR: Negative
09/25 05:30 SARS-CoV-2 rapid antigen: Negative
09/22 CXR: no acute disease
09/22 RUQ US: mild hepatic steatosis, no cirrhosis, no ascites, no gallstones
09/22 EKG: normal sinus rhythm, rate 96, normal intervals

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
09/24 08:00 | 98.4 | 82 | 130/82 | 16 | 98% | RA
09/24 16:00 | 98.6 | 80 | 126/78 | 16 | 98% | RA
09/24 20:00 | 98.3 | 76 | 124/78 | 16 | 97% | RA
09/25 04:00 | 98.1 | 74 | 122/76 | 14 | 98% | RA
09/25 07:30 | 98.2 | 78 | 128/80 | 16 | 98% | RA

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Discharge medication list

DISCHARGE MEDICATION LIST

Patient: Raymond Doe MRN: WB-6613027
All oral. No injectables. No controlled substances.
- Acamprosate 666 mg PO three times daily (started 09/24)
- Thiamine 100 mg PO daily
- Folic acid 1 mg PO daily
- Multivitamin 1 tab PO daily
- Acetaminophen 650 mg PO every 6 hours PRN pain (max 3 g/day)
- Nicotine patch 14 mg/24h daily
30-day supply of all medications filled by inpatient pharmacy (meds to beds) 09/25/2026.

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